

QUICK REFERENCE
FOR HEALTHCARE PROVIDERS



INFLAMMATORY BOWEL DISEASE

2025



Ministry of Health Malaysia



Malaysian Society
of Gastroenterology &
Hepatology (MSGH)



Malaysian Society of
Paediatric Gastroenterology,
Hepatology, and Nutrition
(MySPGHAN)



Academy of Medicine Malaysia

KEY MESSAGES

1. Inflammatory bowel disease (IBD) refers to a group of idiopathic, chronic, and relapsing inflammatory disorders of the gastrointestinal (GI) tract, primarily encompassing ulcerative colitis (UC) and Crohn's disease (CD).
2. The diagnosis of IBD should be established through a comprehensive, integrated assessment encompassing clinical features (including extra-intestinal manifestations), laboratory tests, endoscopic evaluation, histopathological findings, and imaging studies.
3. Colonoscopy in IBD should be performed with the acquisition of at least two biopsy specimens from a minimum of six bowel segments.
4. The approach to IBD management is individualised based on a treat-to-target strategy which include short term, intermediate and long-term goals.
5. A multidisciplinary team approach should be employed to guide decision-making in the management of IBD patients.
6. The medical treatment options include conventional and advanced therapies.
7. Early referral to a colorectal surgeon should be considered in IBD patients requiring surgery.
8. Surveillance endoscopy for dysplasia in IBD patients should begin eight years after symptom onset, while those with concomitant primary sclerosing cholangitis should undergo surveillance from the time of diagnosis with annual endoscopic assessments.
9. Pre-conception counselling should be routinely offered to all women of childbearing age with IBD to optimise disease control prior to conception and to discuss medication safety.
10. Safe and effective management of paediatric IBD should prioritise growth, development and psychosocial well-being.

This Quick Reference provides key messages & a summary of the main recommendations in the Clinical Practice Guidelines (CPG) Management of Inflammatory Bowel Disease.

Details of the evidence supporting these recommendations can be found in the above CPG, available on the following websites:

Ministry of Health Malaysia: www.moh.gov.my

Academy of Medicine Malaysia: www.acadmed.org.my

Malaysian Society of Gastroenterology & Hepatology: www.msgh.org.my

Malaysian Society of Paediatric Gastroenterology, Hepatology and

Nutrition: <https://www.myspghan.my>

International Academy of Pathology Malaysian Division (IAPMD): <https://www.iapmd.net/>

Malaysian Society of Colorectal Surgeons: <https://colorectalmys.org/>

CLINICAL PRACTICE GUIDELINES SECRETARIAT

Malaysian Health Technology Assessment Section (MaHTAS)

Medical Development Division, Ministry of Health Malaysia

Level 4, Block E1, Precinct 1,

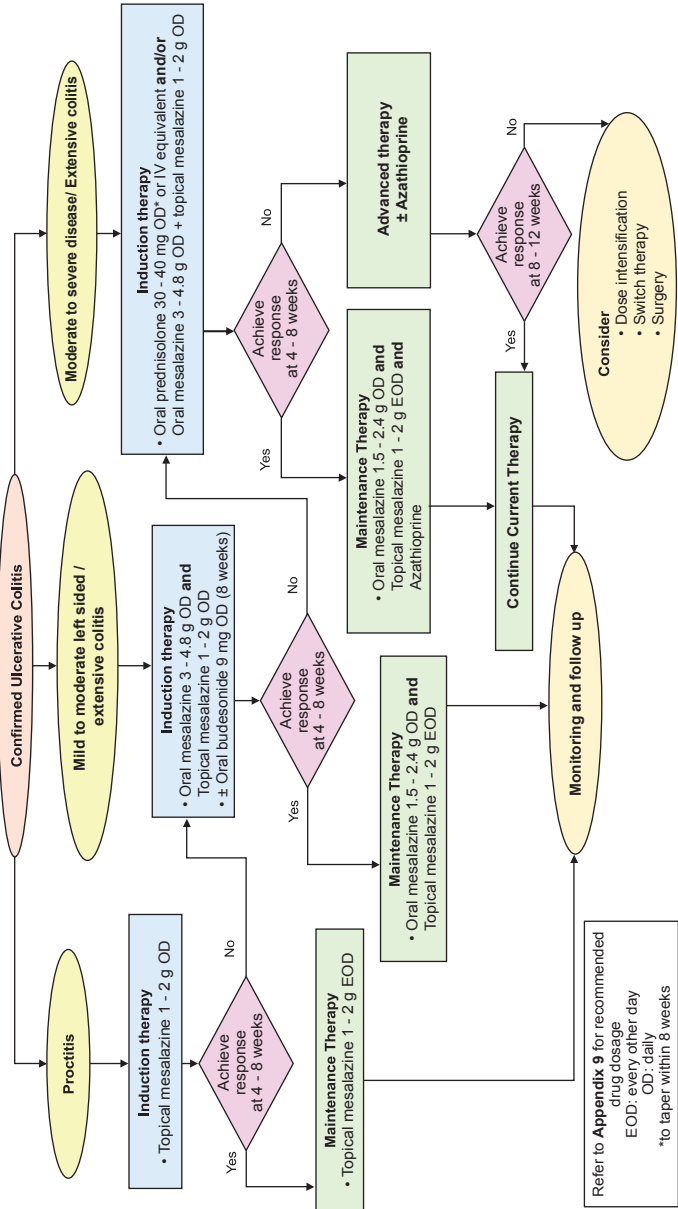
Federal Government Administrative Centre 62590

Putrajaya, Malaysia

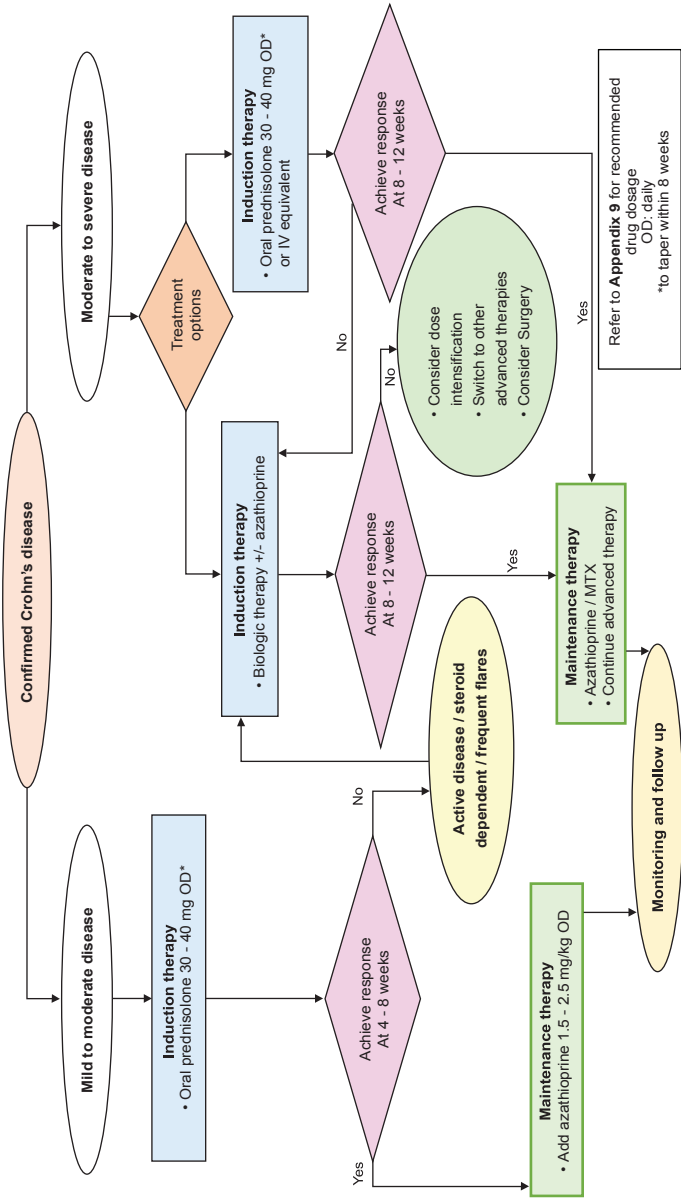
Tel: 603-88831229

E-mail: htamalaysia@moh.gov.my

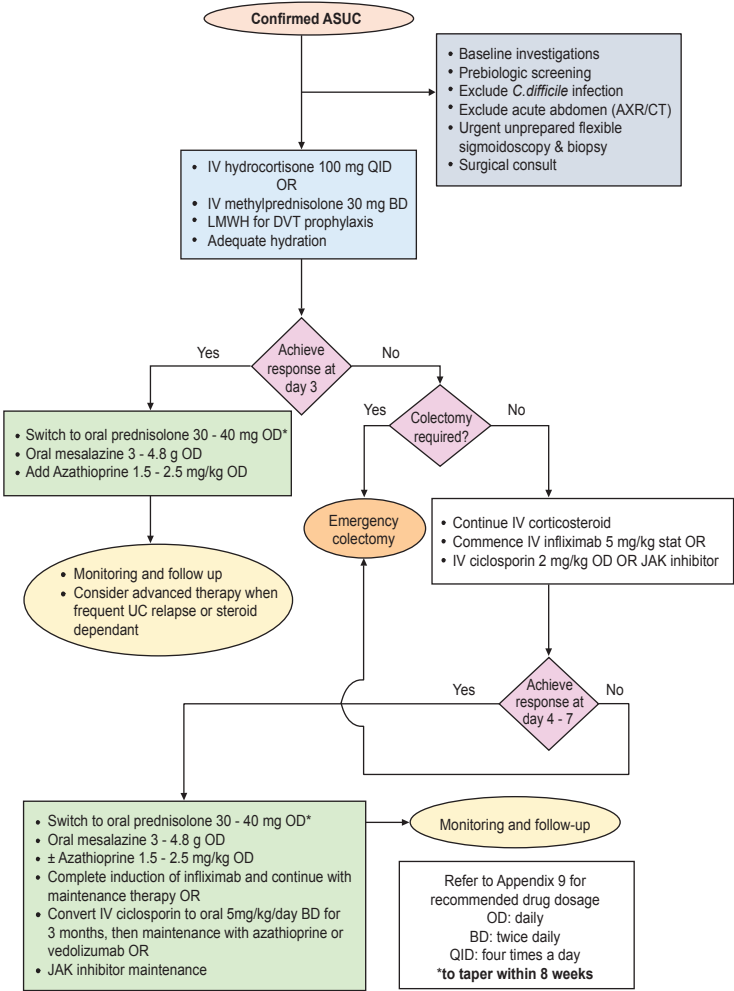
ALGORITHM ON MANAGEMENT OF ULCERATIVE COLITIS



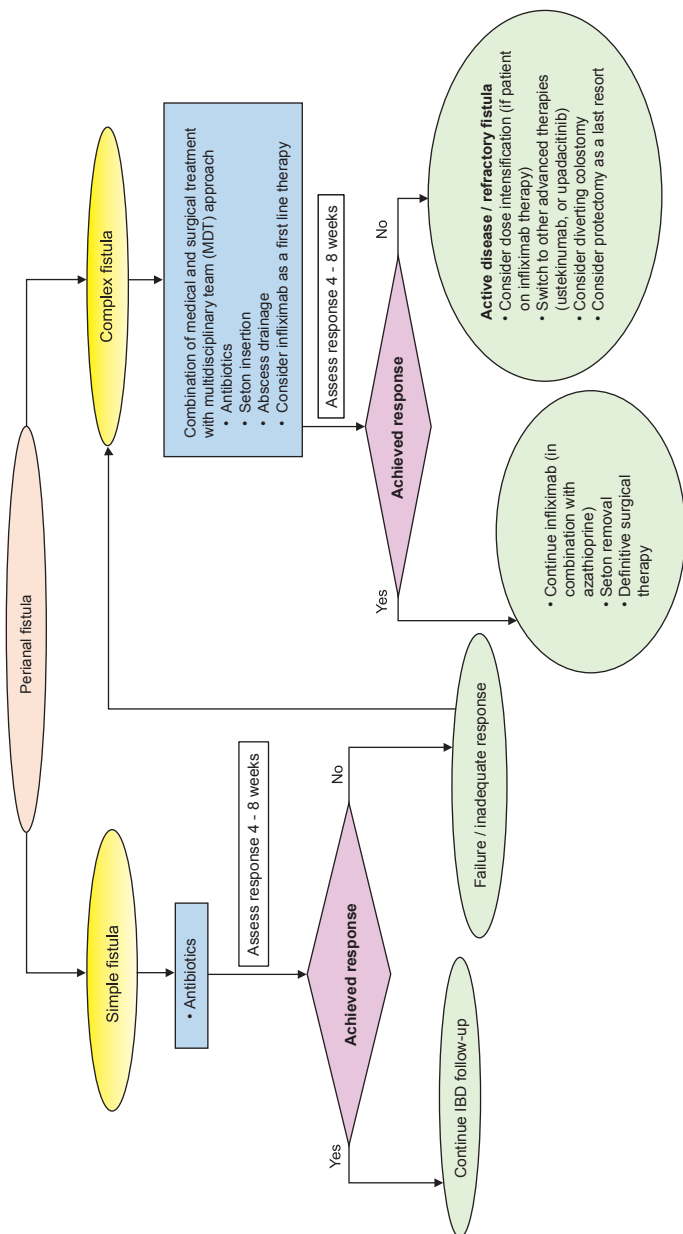
ALGORITHM ON MANAGEMENT OF CROHN'S DISEASE



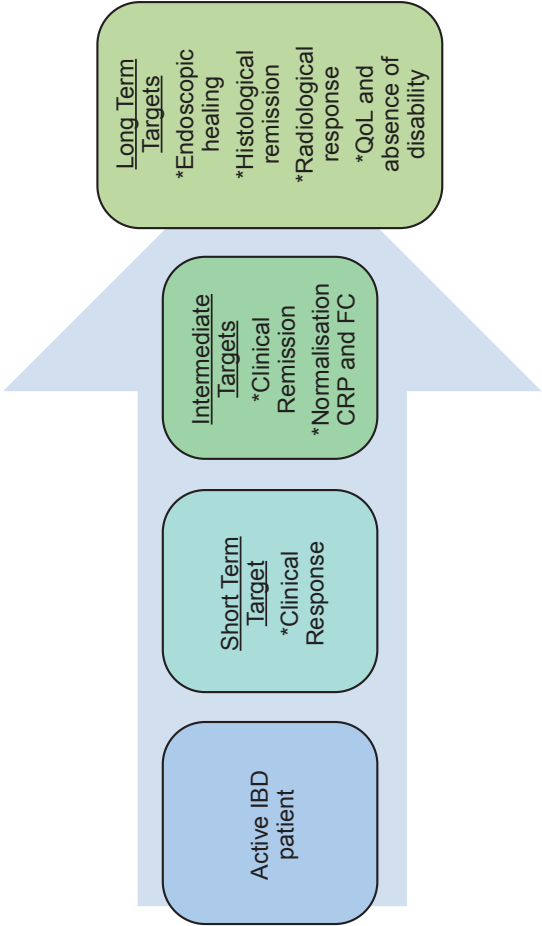
ALGORITHM ON MANAGEMENT OF ACUTE SEVERE
ULCERATIVE COLITIS (ASUC)



ALGORITHM ON MANAGEMENT OF PERIANAL FISTULA



TREATMENT TARGETS FOR PATIENTS WITH IBD



OVERVIEW OF RISK OF DRUGS DURING PREGNANCY AND LACTATION IN IBD

Medication group	Drugs	Pregnancy risk	Lactation risk	Comments
CS	Prednisolone	Low risk	Low risk	Short-term use generally safe; no strong link to birth defects
	Budesonide			
Antibiotics	Metronidazole	Low risk	Avoid	Mainly used in CD patients with pouchitis, perianal disease, or abdominal sepsis
	Ciprofloxacin	Low risk, Avoid in first trimester	Low risk	
Amino salicylates	Mesalazine	Low risk	Low risk	No increased risk of birth defects or major complications.
	Sulphasalazine			Folate supplementation (2 mg/day) is recommended with sulphasalazine to prevent toxicity.
Immunomodulators	Thiopurines	Low risk	Low risk	Generally safe; not linked to birth defects or major AEs
	Ciclosporin, Tacrolimus	Low risk, limited data	Limited data	Limited IBD-specific data; with no clear link to birth defects. May be considered in severe cases, but use requires careful risk-benefit assessment.
	Methotrexate	Contra-indicated	Contra-indicated	Both are teratogenic and strictly contraindicated in pregnancy.
	Thalidomide			Can cause severe birth defects, fetal loss, and high neonatal mortality. Discontinue methotrexate 3 months before conception.
Anti-TNFα	Infliximab Adalimumab	Low risk	Low risk	Not linked to birth defects or poor outcomes; safe as mono or combo therapy.
Anti-α4β7 integrin	Vedolizumab	Low risk, limited data	Low risk	Appear safe , but data are limited; close monitoring recommended.
Anti-IL-12 and anti IL-23	Ustekinumab Risankizumab	Low risk, limited data	Low risk	
JAK inhibitors	Tofacitinib Upadacitinib Filgotinib	Contra-indicated	No data; avoid	Contraindicated due to limited human data and animal studies showing fetal harm.
Sphingosine-1-phosphate receptor modulators	Ozanimod	Contra-indicated	No data; avoid	Safety in pregnancy is unproven; avoid use until more evidence is available.

ADOSING FOR COMMONLY USED MEDICATIONS IN INFLAMMATORY BOWEL DISEASE

DRUG	RECOMMENDED DOSAGE (ADULTS)	RECOMMENDED DOSAGE (PEDIATRICS)
Prednisolone	PO 30-40 mg once daily; taper down 5 mg weekly (duration: ≤ 8 weeks) *High bioavailability with extensive systemic absorption *Strong anti-inflammatory effect with higher risk of AEs	1 mg/kg/day (weight-dependent; max 40 mg/day) once daily for 1-2 weeks (not more than 4 weeks), then taper down by 5 mg weekly (duration: ≤ 8 weeks)
Budesonide	PO 9 mg once daily (duration: ≤ 8 weeks) *Low bioavailability with effective local action in the gut and reduced systemic exposure *Lower risk of AEs compared to prednisolone	NA
Mesalazine (5-ASA)	<u>Oral formulation:</u> - Mesalazine 500 mg MR tablet Induction: PO 3.0-4.0 g/day in divided doses Maintenance: PO 1.5-2.0 g/day in divided dose - Mesalazine 1.2 g gastro-resistant prolonged tablet Induction: PO 3.6-4.8 g/day once daily Maintenance: PO 2.4 g/day once daily - Mesalazine 1.5 g gastro-resistant prolonged granules Induction: PO 3.0-4.5 g once daily Maintenance: PO 1.5-3.0 g once daily - Mesalazine 2 g prolonged granules Induction: PO 4.0 g/day in divided doses Maintenance: PO 2.0 g/day in divided dose <u>Topical formulation:</u> - Mesalazine 1 g suppository: 1 g rectally once daily (retained in the rectum 1 to 3 hours), or - Mesalazine 1 g enema: 1-2 g rectally once daily	<u>Oral formulation:</u> Induction: PO Mesalazine 50 – 70 mg/kg/day up to 100 mg/kg/day or 4.8 g daily + rectal Mesalazine 25 mg/kg up to 1g daily Maintenance: The induction dose that achieves clinical effectiveness may be used for maintenance, with dose reduction as appropriate within the recommended range of 50–70 mg/kg/day <u>Topical formulation:</u> Rectal Mesalazine 25 mg/kg up to 1 g daily
Azathioprine	PO Initially 50 mg once daily; titrate up to 2.5 mg/kg once daily over ≥12 weeks as indicated and tolerated	PO initially 1 mg/kg/day then titrate up to 2 – 2.5 mg/kg/day

DRUG	RECOMMENDED DOSAGE (ADULTS)	RECOMMENDED DOSAGE (PEDIATRICS)
Methotrexate (MTX)	<u>Moderate to severe CD</u> Induction: - IM or SC 15 - 25 mg once weekly (in combination with folic acid), or - PO, IM or SC 12.5 - 15 mg once weekly (in combination with biologic therapy). Maximum 25 mg once weekly Maintenance: IM or SC up to 25 mg once weekly. If remission maintained for 4 months, may reduce to 15 mg once weekly Moderate UC: PO 1-1.5 mg/kg/day Moderate CD: PO 0.75-1.5 mg/kg/day Induction: IV 5 mg/kg at week 0, 2, and 6	PO 15 mg/m² (max 25 mg) once a week + PO Folate 5 mg 24 – 72 hours after MTX once a week
Mercaptopurine (6-MP)	Induction: IV 5 mg/kg at week 0, 2, and 6	NA
Infliximab	Maintenance: IV 5-10 mg/kg every 8 weeks Induction: SC 160 mg at week 0 (either 4 injections in a day, or 2 injections for two days), then 80 mg at week 2, then Maintenance: SC 40 mg every other week	Induction: IV 5 – 10 mg/kg at week 0, 2 and 6 Maintenance: IV 5 – 10 mg/kg every 4 – 8 weeks Induction at week 0: 160 mg (> 40 kg) or 80 mg (< 40 kg) Maintenance: 80 mg week 2 then continue with 40 mg every other week (> 40 kg), or 40 mg week 2 onwards every other week (< 40 kg)
Adalimumab	Induction: IV 300 mg at week 0, 2 and 6 Maintenance: IV 300 mg every 8 weeks	Induction: IV 200 mg/m² or 10 mg/kg (max 300 mg) every 8 weeks from week 14 Maintenance: IV 200 mg/m² or 10 mg/kg (max 300 mg) every 8 weeks from week 14
Vedolizumab	Induction: IV 300 mg at week 0, 2 and 6 Maintenance: IV 300 mg every 8 weeks	Induction: IV 200 mg/m² or 10 mg/kg (max 300 mg) at week 0, 2 and 6 Maintenance: IV 200 mg/m² or 10 mg/kg (max 300 mg) every 8 weeks from week 14 Induction: IV 6 mg/kg rounded 130 mg (max 520 mg)
Ustekinumab	Induction: 55 kg and less: IV 260 mg single dose > 55 kg to 85 kg: IV 390 mg single dose > 85 kg: IV 520 mg Maintenance: - SC 90 mg every 8 weeks (starting 8 weeks after IV induction dose)	Maintenance: BSA adjusted dose SC 90 mg or 45 mg every 8 weeks from week 8
Risankizumab	<u>Moderate to severe CD</u> Induction: IV 600 mg at weeks 0, 4 and 8 Maintenance: SC 180 to 360 mg at week 12, then every 8 weeks thereafter <u>Moderate to severe UC</u> Induction: IV 1200 mg at weeks 0, 4 and 8 Maintenance: SC 180 to 360 mg at week 12, then every 8 weeks thereafter	NA

DRUG	RECOMMENDED DOSAGE (ADULTS)	RECOMMENDED DOSAGE (PEDIATRICS)
Guselkumab	<u>Moderate to severe UC</u> Induction: IV 200 mg at weeks 0, 4, and 8 Maintenance: SC 100 mg every 8 weeks or 200 mg every 4 weeks	NA
*Mirikizumab	<u>Moderate to severe CD</u> Induction: IV 900 mg at weeks 0, 4 and 8 Maintenance: SC 300 mg at week 12, then every 4 weeks dosing <u>Moderate to severe UC</u> Induction: IV 300 mg at weeks 0, 4 and 8 Maintenance: SC 200 mg at weeks 12, then every 4 weeks thereafter <i>*If dose is missed, administer as soon as possible and then resume every 4-week dosing.</i>	NA
Tofacitinib	<u>Moderate to severe UC</u> Induction: PO 10 mg twice daily for at least 8 weeks, maximum 16 weeks or transition of maintenance dose <i>*Discontinue if inadequate response after 16 weeks with PO 10 mg twice daily</i> Maintenance: PO 5 mg twice daily	NA
Upadacitinib	<u>Moderate to severe CD</u> Induction: PO 45 mg once daily for at least 12 weeks Maintenance: PO 15-30 mg once daily <i>*Discontinue if inadequate response with PO 30 mg once daily</i> <u>Moderate to severe UC</u> Induction: PO 45 mg once daily for at least 8 weeks Maintenance: PO 15-30 mg once daily <i>*Discontinue if inadequate response with PO 30 mg once daily</i>	NA
Cidosporin	<u>ASUC</u> Induction: IV 2 mg/kg/day, infused continuously over 24 hours Maintenance: 2.5 mg/kg BD	<u>ASUC</u> IV 2 mg/kg/day, infused continuously over 24 hours up to 7 days or until response

DRUG	RECOMMENDED DOSAGE (ADULTS)	RECOMMENDED DOSAGE (PEDIATRICS)
*Etrasimod	Moderate to severe UC: PO 2mg once daily	NA
*Ozanimod	Moderate to severe UC Induction: PO 0.23 mg once daily (Day 1 - Day 4), then 0.46mg once daily (Day 5 to Day 7) Maintenance: PO 0.92mg once daily (starting Day 8 onwards) <i>*If missed during the first 2 weeks of treatment, re-initiate the titration regimen with 0.23 mg once daily.</i> <i>*If a dose missed after the first 2 weeks of treatment, continue with treatment as planned.</i>	NA

*Unavailable in Malaysia

- AEs - adverse effects
- ASUC - acute severe ulcerative colitis
- CD - Crohn's disease
- IM - intra-muscular
- IV - intra-venous
- NA - not applicable
- PO - per oral
- SC - sub-cutaneous
- UC - ulcerative colitis